



EVALUATION 7 — INCOMPREHENSIBLY COMPLEX ("BREAK THE ADMINISTRATOR")

Primary Diagnosis: Pediatric-to-Adult Transition with Congenital Heart Disease (Single Ventricle Physiology, Status Post Fontan), Heart-Lung Transplant Evaluation, Home Inotrope Infusion, BiPAP, Jejunostomy Feeds, Anticoagulation, Protein-Losing Enteropathy, Plastic Bronchitis, Liver Cirrhosis (Fontan-Associated), Renal Failure on Peritoneal Dialysis, Sickle Cell Trait Interaction, Severe Developmental Delay, Non-Verbal with Augmentative Communication, Legal Conservatorship, and Multi-Agency Care Coordination

PATIENT DEMOGRAPHICS

Field Detail

Patient Name Esperanza "Espie" Milagros Reyes

Date of Birth 09/17/2003

Age 21

Sex Female

Race/Ethnicity Hispanic/Latina (Filipino and Mexican American, multiethnic)

Primary Language Non-verbal — uses AAC device (Tobii Dynavox Snap + Core First); understands English and Tagalog

Interpreter Needed Yes — Tagalog interpreter required when communicating with mother (primary language Tagalog; understands conversational English but not complex medical terminology)

Marital Status Single

Medical Record Number CI-2025-00463

Address of Service 7540 Bayshore Circle, Apt 3A (ground floor, ADA-adapted), Pacifica, CA 94044

Cell Phone N/A (patient non-verbal; mother cell below)

Mother Cell (650) 555-0327

Email <maria.reyes.care@email.com> (mother's email; monitors on patient's behalf)

County San Mateo

LEGAL STATUS

Field Detail

Conservator Maria Santos Reyes (mother) — Limited conservatorship granted 09/2021 by San Mateo County Superior Court (Case # _____). Conservatorship covers: medical decision-making, financial decisions, residence decisions, and education decisions.

Conservatorship Documentation Letters of Conservatorship on file (copy in clinical record).

Capacity Patient does not have decision-making capacity for complex medical decisions due to moderate-severe intellectual disability (IQ estimated 35-40 per neuropsychological evaluation 2022). Patient can express basic preferences (food, comfort, like/dislike, pain) via AAC device and facial expression. Patient demonstrates understanding of simple explanations but cannot comprehend complex risk/benefit analyses or provide informed consent. All consents signed by conservator (mother).

Regional Center Golden Gate Regional Center — Service Coordinator: Sandra Iglesias, (650) 555-0991

IHSS Active — 283 hours/month authorized (mother is paid IHSS provider)

SSI/SSDI SSI recipient — Representative Payee: Maria Santos Reyes

EMERGENCY CONTACTS

Priority Name Relationship Phone Lives with Patient?

1 Maria Santos Reyes Mother / Conservator (650) 555-0327 (cell) / (650) 555-0328 (home) Yes

2 Ricardo Reyes Father (650) 555-0412 Yes (parents married, both in home)

3 Isabella Reyes Sister (age 17 — trained caregiver, high school senior) (650) 555-0329 Yes

4 Lucene Heart-Lung Transplant Coordinator Stanford (650) 555-1675 (24-hour transplant line) N/A

5 Filipino Community Health Worker Gawad Kalinga Clinic (650) 555-0850 N/A — community navigator for family

Other persons in home: Grandmother (Lola Consuelo, age 78, dementia — patient's grandmother requires some supervision herself; lives in home; complicates caregiver burden)

LIVING SITUATION

Field Detail

Lives With Mother Maria (52), Father Ricardo (55, works construction — away 10-12 hrs/day), Sister Isabella (17 — helps after school/weekends), Grandmother Consuelo (78, mild-moderate dementia — requires supervision)

Residence 3BR ground-floor ADA-adapted apartment; wheelchair accessible; roll-in shower; widened doorways

DME (see extensive DME section below)

Pets 1 small dog ("Bituin," Pomeranian, 6 lbs) — emotional support animal; patient's primary comfort; non-allergenic food only near patient

Cultural/Religious Devout Catholic; faith is central to family coping. Weekly communion brought to home by parish eucharistic minister.

Cultural integration of Filipino healing practices (hilot — traditional massage; mother applies to patient's extremities; not harmful, may be beneficial for comfort/circulation). Holy water and religious items at bedside per family custom — accommodate.

DME INVENTORY

Equipment Details

BiPAP ResMed AirCurve 10 VAuto; settings: IPAP 18, EPAP 8, Rate 14; used during sleep (10 PM – 7 AM) and during daytime naps; patient NOT vent-dependent but requires NIV for sleep and periods of fatigue/decompensation

Pulse Oximeter Continuous overnight; spot-check Q4H daytime; alarm set at <88%

Suction Machine Portable DeVilbiss; for oral suctioning (excessive secretions — plastic bronchitis)

Cough Assist Philips CoughAssist E70; settings: +40/-40 cmH2O; used BID + PRN for cast/plug expectoration (plastic bronchitis)

Nebulizer PARI LC Plus; for inhaled medications

O2 Concentrator 5 LPM concentrator; PRN for desaturation (SpO2 <88%); patient not on continuous O2 at baseline

Infusion Pump (Milrinone) CADD-Solis ambulatory pump; milrinone infusion via Hickman catheter 24/7; THIS IS A CONTINUOUS, LIFE-SUSTAINING INFUSION — abrupt discontinuation can cause acute decompensation and death

Peritoneal Dialysis Cycler Baxter HomeChoice Claria; nightly automated PD (10 PM – 7 AM); dwells programmed per nephrology

J-Tube Feeding Pump Kangaroo ePump; continuous feeds 18 hrs/day

Hospital Bed Full-electric with alternating pressure mattress

Wheelchair Permobil M3 Corpus tilt-in-space; head-array drive control (patient cannot use hands for joystick); custom seating

Hoyer Lift Portable (ceiling track not feasible in apartment)

Shower/Commode Chair Tilt-in-space shower chair

AAC Device Tobii Dynavox I-Series with Snap + Core First; eye-gaze access; mounting arm on wheelchair; secondary device: iPad with TouchChat

Cardiac Monitor Portable telemetry (event monitor) — for arrhythmia detection; transmitted daily to cardiology

Specialty Supplies PD supplies (delivered monthly — Baxter), Hickman catheter supplies, J-tube supplies, wound care supplies, cast expulsion supplies (specimen cups for bronchial casts), emergency medication kit

PAYER INFORMATION

Priority Payer ID / Policy Number

Primary Medi-Cal Managed Care (Health Plan of San Mateo — CCS transition) HPSM-90518833E

Secondary California Children's Services (CCS) — transitioning to CCS Whole Child Model via HPSM CCS-SM-2025-07741

Tertiary SSI/Medi-Cal Linked

Note: This patient is transitioning from pediatric to adult services. CCS has historically covered all cardiac-related care. Under the CCS Whole Child Model, HPSM now coordinates CCS and non-CCS services through a single managed care plan. Prior authorizations are complex — some services route through CCS carve-out, others through HPSM directly. Agency billing team must coordinate closely with HPSM CCS care coordinator for authorization of all services.

CCS Care Coordinator: Jennifer Lam, (650) 555-0744 HPSM Care Manager: Patricia Okonkwo, RN, (650) 555-0288

PHYSICIAN INFORMATION (9 specialists + PCP)

Role Name Specialty Phone

Attending / Certifying Dr. Michael Chen Adult Congenital Heart Disease (ACHD) (650) 555-1700

Heart-Lung Transplant Dr. Elizabeth Hartfield Cardiothoracic Surgery / Transplant (650) 555-1675

Pulmonologist Dr. Sarah Atkins Pulmonary (650) 555-1135

Nephrologist Dr. Amir Hussain Nephrology (Peritoneal Dialysis) (650) 555-1920

Hepatologist Dr. Michelle Park Hepatology (650) 555-1605

GI Dr. Heather Kim Gastroenterology (650) 555-1320

Hematologist Dr. Laverne Okafor Hematology (anticoagulation + sickle trait) (650) 555-1830

Developmental Medicine Dr. Priya Mehta Developmental-Behavioral Pediatrics (transitioning to adult) (650) 555-1122

Psychiatrist Dr. Nina Kovacs Psychiatry (650) 555-1780

PCP Dr. Evelyn Nakamura Family Medicine (650) 555-0400

F2F Encounter Date: 06/06/2025 (ACHD clinic visit) F2F Performed By: Dr. Michael Chen

REFERRAL / HOSPITALIZATION HISTORY (extensive — lifelong cardiac patient)

Event Facility Dates Reason

Most Recent Admission Lucile Packard / Stanford 05/10/2025 – 06/08/2025 Acute decompensated heart failure (Fontan failure); started milrinone infusion; Hickman placed for home inotrope; initiated heart-lung transplant evaluation; plastic bronchitis exacerbation requiring bronchoscopy x2; PD catheter placed (Fontan-associated renal failure)

Previous Admission Lucile Packard 02/2025 Plastic bronchitis — respiratory distress, bronchoscopy, ICU admission

Previous Admission Lucile Packard 10/2024 PLE flare — severe hypoalbuminemia (albumin 1.2), massive edema, IV albumin replacement

Previous Admission Lucile Packard 06/2024 Atrial flutter with rapid ventricular response; cardioversion; amiodarone started

Surgical / Procedural History (cardiac — lifelong):

Neonatal: Norwood Stage I (2003)

Infancy: Bidirectional Glenn (2004)

Age 3: Fontan completion — extracardiac conduit (2006)

Age 12: Fontan revision with fenestration (2015)

Age 16: PLE diagnosed; initiated budesonide and heparin protocol (2019)

Age 18: Pacemaker implantation — epicardial, dual-chamber (2021) for sick sinus syndrome

Age 21: Hickman catheter placement, PD catheter placement (2025)

Multiple cardiac catheterizations (7 total through 2025)

Multiple bronchoscopies (4 total — plastic bronchitis cast removal)

Jejunostomy placement (2022) for enteral nutrition (oral feeding insufficient)

DIAGNOSES (38+ active diagnoses — most complex patient in this set)

Priority ICD-10 Diagnosis

Primary Q23.4 Hypoplastic left heart syndrome (single ventricle physiology)
Active Z95.812 Status post Fontan procedure (extracardiac conduit, 2006; revised with fenestration, 2015)
Active I50.814 Right heart failure (Fontan failure — decompensated; on home milrinone)
Active Z94.89 Status of heart-lung transplant evaluation (listed/evaluating — UNOS status per transplant team)
Active Z45.09 Encounter for adjustment of cardiac device — pacemaker (epicardial, dual-chamber; battery check Q6 months)
Active I48.3 Typical atrial flutter (history — cardioverted; on amiodarone)
Active I49.5 Sick sinus syndrome (pacemaker-dependent — paced rhythm)
Active Z95.0 Presence of cardiac pacemaker
Active K90.89 Other intestinal malabsorption — protein-losing enteropathy (PLE) — Fontan-associated
Active E44.0 Moderate protein-calorie malnutrition (albumin 2.0; prealbumin 8)
Active J98.09 Other diseases of bronchus, NEC — plastic bronchitis (Fontan-associated; recurrent cast formation; requires CoughAssist + PRN bronchoscopy)
Active J96.11 Chronic respiratory failure with hypoxia (baseline SpO2 88-92% on RA)
Active K76.7 Hepatorenal syndrome (may be more accurately coded as Fontan-associated liver disease)
Active K74.69 Other cirrhosis of liver — Fontan-associated liver disease (FALD); congestive hepatopathy; MELD-XI 18; portal hypertension; esophageal varices (Grade 1, not yet banded)
Active N18.5 Chronic kidney disease, stage 5 (GFR 12 — on peritoneal dialysis)
Active Z99.2 Dependence on renal dialysis (automated peritoneal dialysis, nightly)
Active Z99.81 Dependence on supplemental oxygen (PRN — not continuous)
Active Z93.4 Jejunostomy status (J-tube — primary nutrition route)
Active D57.3 Sickle cell trait (Hb AS) — clinically relevant due to chronic hypoxemia; risk of sickling events at SpO2 <85%; maintain SpO2 ≥88% at all times
Active Z79.01 Long-term anticoagulant use (warfarin — Fontan circuit; enoxaparin bridge PRN)
Active I82.891 Acute embolism and thrombosis — Fontan circuit thrombosis history (2020); on warfarin
Active E87.1 Hyponatremia (chronic — dilutional; Na 129)
Active E87.6 Hypokalemia (PD-related losses)
Active E83.42 Hypomagnesemia
Active E55.9 Vitamin D deficiency
Active D63.1 Anemia in chronic kidney disease (Hgb 8.8; on darbepoetin)
Active F72 Severe intellectual disability (IQ estimated 35-40; non-verbal; requires full assistance with all ADLs)
Active F84.0 Autism spectrum disorder (diagnosed age 5; non-verbal; stereotypic behaviors — hand-flapping, rocking, which increase when distressed or in pain)
Active F31.32 Bipolar II disorder, current episode depressed, moderate (diagnosed 2023; manifests as periods of agitation, self-injurious behavior — head-hitting, skin-picking — alternating with withdrawal/anhedonia)
Active R45.89 Other symptoms and signs involving emotional state — episodic agitation and self-injurious behavior
Active G40.909 Epilepsy, unspecified, not intractable (seizure history — last seizure 2024; on levetiracetam)
Active L89.152 Pressure ulcer, sacral region, Stage 2 (2.0 cm x 1.5 cm, granulating)
Active L89.019 Pressure ulcer, right elbow, unstageable (eschar — dry, stable, intact; do not debride per wound care MD — monitoring only)
Active R62.52 Short stature (due to chronic illness; 4'9", 78 lbs)
Active E46 Unspecified protein-calorie malnutrition
Active R63.6 Underweight (BMI 13.1 — severely underweight)
Active K22.10 Esophageal varices without bleeding (Grade 1 — Fontan portal HTN)
Active R00.1 Bradycardia (pacemaker-managed)

ALLERGIES

Allergen Reaction Severity

Heparin Heparin-induced thrombocytopenia (HIT) Type II — confirmed 2020 LIFE-THREATENING — NO HEPARIN IN ANY FORM (including flushes, coated catheters, line priming). Use NS flushes only. Argatroban for anticoagulation bridge if IV needed.
Morphine Respiratory depression (near-intubation event, 2019) SEVERE
Amoxicillin Serum sickness-like reaction (fever, rash, joint pain) Severe
Latex Anaphylaxis LIFE-THREATENING
Iodine contrast Severe urticaria + bronchospasm (required epinephrine 2021) Severe
Eggs Angioedema Moderate — relevant to enteral formula selection
Soy GI intolerance (cramping, diarrhea) Moderate — relevant to enteral formula selection
Formula note: Patient requires egg-free, soy-free enteral formula. Current: Peptamen 1.5 (Nestle) — semi-elemental, egg-free, soy-free; tolerated. Confirmed with GI/RD.

CURRENT MEDICATIONS (42 medications — maximum complexity)

#	Medication	Dose	Route	Frequency	Purpose	Critical	Notes
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- 1 Milrinone 0.375 mcg/kg/min IV continuous via Hickman 24/7 Inotropic support (Fontan failure) LIFE-SUSTAINING. DO NOT DISCONTINUE. Abrupt cessation → acute hemodynamic collapse. CADD pump; pharmacy-prepared bags delivered weekly. Prime/load/rate change per specific protocol.
- 2 Warfarin 3 mg PO (via J-tube, crushed, suspended in water) Daily (5 PM) Anticoagulation (Fontan circuit) INR target 2.5-3.5 (higher target due to Fontan). NO HEPARIN — HIT history. NS flushes only. Draw INR 2x/week.
- 3 Amiodarone 200 mg PO (via J-tube) Daily Atrial flutter prophylaxis Monitor thyroid (TSH Q3 months), LFTs (already abnormal — Fontan liver; coordinate with hepatology), pulmonary (PFTs if feasible), corneal (ophthalmology annual). Interacts with warfarin (increases INR — dose stabilized but monitor closely).
- 4 Budesonide (Entocort) 9 mg PO (via J-tube — do NOT crush capsule contents further; open capsule and pour granules into J-tube with water flush) Daily AM Protein-losing enteropathy (PLE) Part of PLE management triad (budesonide + albumin + heparin-alternative); monitor for adrenal suppression, infection risk.
- 5 Spironolactone 25 mg PO (via J-tube) BID Diuresis; Fontan failure; ascites; hepatic Monitor K⁺ (risk of hyperkalemia with CKD5 + spironolactone + PD); hold if K⁺ >5.5
- 6 Furosemide 40 mg PO (via J-tube) BID Diuresis; volume overload Monitor electrolytes; adjust with PD adequacy
- 7 Sildenafil (Revatio) 20 mg PO (via J-tube) TID Pulmonary vascular resistance reduction (Fontan optimization) Monitor BP; avoid nitrates.
- 8 Bosentan (Tracleer) 62.5 mg PO (via J-tube) BID Pulmonary vasodilator (Fontan optimization) Monthly LFTs required (hepatotoxicity — already compromised liver); pregnancy prevention required (teratogenic — patient has conservatorship, verify contraception is addressed).
- 9 Levetiracetam (Keppra) 500 mg PO (via J-tube) BID Seizure prophylaxis Monitor for behavioral side effects (may worsen agitation in ASD/ID population).
- 10 Risperidone 1 mg PO (via J-tube) BID Agitation, SIB (self-injurious behavior), bipolar Monitor for metabolic effects (weight, glucose); QTc monitoring (already on amiodarone — additive QT prolongation concern); tardive dyskinesia screening Q6 months.
- 11 Sertraline 75 mg PO (via J-tube) Daily AM Depression / anxiety / ASD-related irritability Drug interaction with amiodarone (QT).
- 12 Darbepoetin alfa (Aranesp) 40 mcg SubQ Every 2 weeks Anemia (CKD) Target Hgb 10-11; monitor iron studies.
- 13 IV Albumin 25% 50 mL (12.5 g) IV via Hickman 2x/week (Mon, Thu) PLE — albumin replacement Infuse over 2 hours. Monitor for fluid overload (BiPAP + PD night may mitigate).
- 14 Peritoneal Dialysis Per nephrologist prescription (4 exchanges, 1.5% dextrose, 2000 mL dwell volume) PD catheter Nightly (automated cycler 10 PM – 7 AM) ESRD / CKD5 PD and BiPAP run simultaneously overnight. Train caregivers on setup/disconnect for both. Monitor exit site for peritonitis signs. PD effluent: clear/straw — report if cloudy (peritonitis).
- 15 Enteral formula (Peptamen 1.5) 50 mL/hr J-tube (continuous pump) 18 hrs/day (6 AM – 12 AM) Primary nutrition Egg-free, soy-free, semi-elemental. 1350 kcal/day + modular protein. Free water flushes 150 mL Q4H.
- 16 Modular protein powder 6 g Via J-tube (mixed in water flush) QID Additional protein (PLE losses)
- 17 Pancrelipase (Creon) 24,000 units PO/via J-tube With formula initiation and Q6H Fat malabsorption (PLE) Swallow capsule contents — do not crush microspheres
- 18 Ursodiol 300 mg PO (via J-tube) TID Fontan-associated liver disease
- 19 Lactulose 15 mL PO (via J-tube) BID (titrate to 2-3 soft BMs/day) Hepatic encephalopathy prophylaxis (Fontan liver)
- 20 Rifaximin (Xifaxan) 550 mg PO (via J-tube) BID Hepatic encephalopathy prophylaxis (adjunct) Monitor LFTs
- 21 Albuterol 2.5 mg/3 mL INH (nebulizer) Q4H + PRN Bronchospasm, secretion mobilization (plastic bronchitis)
- 22 Hypertonic Saline 3% 4 mL INH (nebulizer) BID Mucolytic — plastic bronchitis cast prevention/mobilization Monitor SpO₂ during; may trigger bronchospasm — give albuterol first
- 23 Dornase alfa (Pulmozyme) 2.5 mg INH (nebulizer) Daily Mucolytic — plastic bronchitis
- 24 N-Acetylcysteine (Mucomyst) 10% 3 mL INH (nebulizer) BID Mucolytic — plastic bronchitis (adjunct) Bronchospasm risk — give after albuterol
- 25 Pantoprazole 40 mg PO (via J-tube) Daily GI prophylaxis; variceal protection
- 26 Sucralfate 1 g PO (via J-tube) QID (1 hr before meals/feeds — schedule around J-tube feeds) GI mucosal protection (varices) Interacts with many meds — give 2 hrs apart from warfarin, levothyroxine, etc.
- 27 Calcium carbonate 500 mg PO (via J-tube) TID with feeds Phosphorus binder (CKD) + calcium supplement
- 28 Cholecalciferol (Vit D3) 50,000 IU PO (via J-tube) Weekly Vitamin D deficiency (CKD + malabsorption)
- 29 Folic acid 1 mg PO (via J-tube) Daily Anemia support
- 30 Ferrous sulfate (liquid) 15 mg elemental Fe PO (via J-tube) Daily (give 2 hrs apart from PPI, calcium, and sucralfate) Iron deficiency
- 31 Magnesium oxide 400 mg PO (via J-tube) BID Hypomagnesemia
- 32 Potassium chloride (liquid) 20 mEq PO (via J-tube) Daily (adjust per labs) Hypokalemia Careful — spironolactone + PD may swing K⁺ both ways
- 33 Sodium chloride tabs 1 g PO (via J-tube) TID Hyponatremia
- 34 Zinc sulfate 220 mg PO (via J-tube) Daily Wound healing + PLE losses
- 35 Vitamin A 10,000 IU PO (via J-tube) Daily Fat-soluble vitamin deficiency (malabsorption) Monitor levels — hepatotoxicity risk in liver disease
- 36 Vitamin E 400 IU PO (via J-tube) Daily Fat-soluble vitamin deficiency
- 37 Vitamin K1 (phytonadione) 5 mg PO (via J-tube) Weekly (separate day from warfarin dose change) Fat-soluble vitamin deficiency + liver coagulopathy BUT on warfarin — dose carefully balanced Vitamin K antagonizes warfarin; dose is precisely balanced by hematology — do NOT adjust without MD order

38 Multivitamin (liquid, pediatric formulation) 5 mL PO (via J-tube) Daily General Pediatric formulation due to patient size and J-tube tolerance

39 Melatonin 3 mg PO (via J-tube) HS Sleep regulation (ASD)

40 Acetaminophen (liquid) 325 mg PO (via J-tube) Q6H PRN Pain / fever Reduced dose — liver disease. Max 2g/day. NO NSAIDs (varices, CKD, anticoagulation).

41 Chlorhexidine oral rinse 0.12% 15 mL swish-and-spit Oral BID Oral hygiene (immunocompromised, on inotropes — endocarditis prophylaxis adjunct) Mother assists; patient cannot swish and spit independently — apply with oral swabs

42 Emergency medications Epinephrine auto-injector (EpiPen Jr) — for anaphylaxis (multiple severe allergies); Diazepam rectal gel (Diastat) 10 mg — for seizure >5 min IM / PR PRN EMERGENCY Anaphylaxis; status epilepticus Must be immediately accessible at all times. All caregivers trained.

TOTAL MEDICATIONS: 42 IV Medications: 2 (milrinone continuous, albumin 2x/week) Controlled Substances: Diazepam rectal gel (emergency only) Hickman Catheter: Dual-lumen; Lumen 1: milrinone continuous; Lumen 2: albumin infusions, labs. NO HEPARIN FLUSHES — HIT HISTORY. NS FLUSHES ONLY. PD Catheter: Tenckhoff, left lower abdomen. Exit site care per nephrology protocol. J-Tube: Mic-Key low-profile 14 Fr jejunostomy.

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter Value Notes

BP 88/52 Baseline for Fontan patient on milrinone — hypotension is expected. Contact ACHD if SBP <80 or symptomatic.

HR 72 (paced rhythm — VVIR) Pacemaker-dependent

RR 24 Mildly elevated — baseline for this patient (restrictive lung disease)

Temp 98.6°F WNL

SpO2 89% on RA Baseline for Fontan patient (cyanotic at baseline). Alarm at <85%. Sick cell trait — maintain ≥88% to avoid sickling.

Weight 78 lbs (35.4 kg) Severely underweight; BMI 13.1

Height 4'9" (144.8 cm) Short stature (chronic illness)

Pain Unable to self-report; FLACC Scale: 4/10 (occasional grimacing, restlessness, intermittent guarding of abdomen) Use FLACC or r-FLACC (revised for cognitively impaired) at each visit

Blood Glucose 92 mg/dL WNL

HOMEBOUND STATUS

CONFIRMED — profoundly homebound. Non-ambulatory (wheelchair-dependent, head-array control). Connected to milrinone infusion pump 24/7. Connected to PD cycler 10 hrs/day. Connected to J-tube pump 18 hrs/day. Requires BiPAP for sleep. Non-verbal with severe intellectual disability — unable to manage self in any community setting. Leaving home requires wheelchair-accessible transport, portable vent/BiPAP, infusion pump, suction, pulse oximeter, emergency medication kit, and a trained attendant capable of managing all devices and responding to emergencies. Taxing effort is extreme. Absences limited to Stanford ACHD clinic visits (monthly) and emergency department visits.

ADVANCE DIRECTIVES

Document Status

AHCD Executed by conservator (mother Maria) — 06/2025. Maria designated as sole decision-maker. Specific instructions: Full aggressive treatment including intubation, mechanical ventilation, CPR, transplant if offered. Maria states: "She is a fighter. She has survived everything. God will decide when it's her time, not us."

POLST Full treatment (all sections).

Transplant Status Patient is being evaluated for heart-lung transplant at Stanford. If listed and organ offer received, agency will be notified by transplant coordinator; patient will be transported emergently to Stanford for transplant surgery.

SERVICES ORDERED

Service Discipline Frequency Duration

Skilled Nursing RN Daily x 4 weeks (IV milrinone management, albumin infusions, Hickman care, PD oversight, J-tube management, wound care, labs, teaching, respiratory treatments); then 5x/week ongoing Indefinite

Physical Therapy PT 3x/week Indefinite

Occupational Therapy OT 3x/week Indefinite

Speech-Language Pathology SLP 2x/week (AAC optimization, oral-motor, swallow safety) Indefinite

Medical Social Work MSW 1x/week Indefinite

Home Health Aide HHA BID (AM + PM — personal care, repositioning, equipment assist) Indefinite

Respiratory Therapy RT (if available; otherwise RN + DME RT) 2x/week (CoughAssist, nebulizer protocol, BiPAP compliance) Indefinite

Registered Dietitian RD Biweekly (J-tube formula optimization, PLE nutritional management) Indefinite

SKILLED NURSING FOCUS AREAS — SUMMARY (full detail would require its own sub-document)

Milrinone Infusion Management — Verify pump rate, bag volume, line integrity, Hickman patency EVERY visit. Teach caregiver bag changes.

Emergency: if pump fails → do NOT abruptly stop → manually maintain drip if possible → call transplant coordinator + 911 immediately.

Cardiac monitor data transmission daily.

Hickman Catheter Care — Exit site assessment, dressing change weekly (NO heparin — NS flush only), blood draws via line (Lumen 2, per protocol), CLABSI prevention bundle.

PD Management — Assess PD catheter exit site (peritonitis signs: cloudy effluent, abdominal pain, fever). Record PD volumes (infused vs. drained = ultrafiltration). Coordinate with nephrology for prescription adjustments. Teach caregiver troubleshooting (alarms, drainage failure, leak).

Plastic Bronchitis Management — Aggressive pulmonary toilet: nebulizer schedule (albuterol → hypertonic saline → dornase alfa → Mucomyst → NS; spaced Q2-4H), CoughAssist BID + PRN, suctioning PRN. Monitor for cast expectoration — patient may cough up white/grey rubbery bronchial casts. Save casts in specimen cups for MD review. If cast obstructs airway → suction → CoughAssist → if persistent respiratory distress → call 911 (may need emergent bronchoscopy).

Fontan Failure / Cardiac Monitoring — Daily weight, I&O, BP, HR, SpO2, edema assessment, ascites assessment (abdominal girth), JVD, hepatojugular reflux, peripheral perfusion. Report: weight gain >1 lb/day, new/worsening edema, SpO2 <85%, SBP <80, new arrhythmia symptoms.

PLE Management — Albumin infusion 2x/week per protocol. Monitor albumin levels weekly (target >2.5). Assess edema. J-tube protein supplementation compliance. Coordinate with GI/ACHD.

Liver Disease Monitoring — LFTs per schedule. Assess for hepatic encephalopathy (confusion beyond baseline, sleep-wake reversal, asterix — limited assessment in non-verbal patient; rely on behavioral changes reported by mother). Lactulose/rifaximin compliance. Monitor for variceal bleeding (hematemesis, melena, hematochezia — report immediately).

Anticoagulation — Warfarin INR 2x/week (target 2.5-3.5). Extremely complex drug interaction profile — amiodarone, budesonide, ursodiol, sucralfate, Vitamin K, enteral feeds ALL affect INR. Any medication change → anticipate INR shift → increase monitoring. NO HEPARIN — HIT.

Sickle Cell Trait Monitoring — Maintain SpO2 ≥88%. If SpO2 drops <85% despite O2 supplementation → call 911 (sickling crisis risk). Ensure adequate hydration. Avoid extreme temperature exposure.

Seizure Management — Levetiracetam compliance. Seizure precautions: padded bed rails, nothing in mouth during seizure, time seizure, Diastat if >5 min, call 911. Document and report all seizure activity.

Wound Care — Sacral Stage 2: wound care per orders. Right elbow unstageable (eschar): monitor only, do not debride per wound care MD. Comprehensive skin assessment every visit. Braden 10 (very high risk). Repositioning Q2H.

Behavioral/Psychiatric — Monitor for SIB (self-injurious behavior), agitation, withdrawal. Use behavioral tracking log. Coordinate with psychiatrist. Risperidone monitoring (QTc, metabolic, tardive dyskinesia). Environmental modifications for sensory needs (ASD — low lighting, low noise, weighted blanket, comfort items, dog "Bituin" at bedside).

Multi-Agency Coordination — This patient has: home health (CI), IHSS, Regional Center, CCS, HPSM care manager, transplant team, PD supply vendor (Baxter), infusion pharmacy (Coram), DME vendor (multiple), and 10 physicians. RN serves as central care coordinator. Maintain communication log. Monthly care conferences (virtual) with all providers recommended.

Caregiver Support — Mother Maria is carrying an extraordinary burden: conservator, IHSS provider, primary caregiver for a medically catastrophic 21-year-old AND a 78-year-old with dementia. Father works 10-12 hrs/day. Sister Isabella is 17 and helps but has her own life. Caregiver collapse is a critical and imminent risk. Priorities: (a) maximize IHSS hours; (b) Regional Center respite services; (c) explore adult day health for Espie (medically complex — limited options); (d) explore adult day program or in-home attendant for grandmother; (e) connect Maria with caregiver support group (ideally Filipino community or medically complex child/adult caregiver group); (f) assess Maria's physical and mental health every visit; (g) cultural competence — respect Maria's faith-based coping and Filipino cultural practices.

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

Goal Target

- 1 Zero milrinone interruptions or infusion-related adverse events 08/09/2025
- 2 Zero CLABSI or Hickman complications 08/09/2025
- 3 Zero peritonitis episodes; PD adequate (weekly Kt/V ≥ 1.7) 08/09/2025
- 4 Albumin improved to ≥ 2.3 (from 2.0) 08/09/2025
- 5 Weight stable or gaining (target ≥ 80 lbs) 08/09/2025
- 6 Zero plastic bronchitis episodes requiring hospitalization 08/09/2025
- 7 SpO2 maintained $\geq 88\%$ for $\geq 95\%$ of monitored time 08/09/2025
- 8 INR within target (2.5-3.5) for $\geq 70\%$ of draws 08/09/2025
- 9 Zero seizure events 08/09/2025
- 10 Sacral wound improved (Stage 1 or resolved) 08/09/2025
- 11 Right elbow eschar stable (no deterioration) 08/09/2025
- 12 SIB episodes reduced by $\geq 50\%$ from baseline (behavioral tracking) 08/09/2025
- 13 Patient uses AAC device for ≥ 10 functional communication exchanges per day 08/09/2025
- 14 All caregivers (mother, father, sister) demonstrate competence in: milrinone pump, Hickman care, PD setup/disconnect, J-tube management, CoughAssist, BiPAP, seizure protocol, emergency medications 07/15/2025
- 15 IHSS hours increased to maximum authorized 07/31/2025
- 16 Respite services activated for mother 07/31/2025
- 17 Transplant evaluation progressing; all required pre-transplant workup completed 08/09/2025
- 18 Zero ED visits, zero unplanned hospitalizations 08/09/2025
- 19 Grandmother Consuelo's care needs assessed and addressed (separate plan) 07/31/2025
- 20 Monthly multi-agency care conference established 07/01/2025

PATIENT / CONSERVATOR SIGNATURES — EVALUATION 7

Conservator Signature: _____ Date: _____

Print Name: Maria Santos Reyes

Relationship: Mother / Limited Conservator (San Mateo County Superior Court Case # _____)

Legal Authority: Letters of Conservatorship — copy on file

(Patient unable to sign — non-verbal, severe intellectual disability; conservator signature constitutes legal consent for all services)

Interpreter Used: ☐ Yes — Tagalog telephone interpreter for all complex medical discussion

Clinician Signature: _____ Date: _____

Print Name / Title / Credentials: _____

END OF 7 PATIENT EVALUATIONS

ACUITY PROGRESSION SUMMARY

Eval # Patient Primary Dx Meds Disciplines Key Complexity Drivers

1 Finnegan CHF Class III 15 SN, PT, OT, MSW, HHA Multi-system failure, polypharmacy (warfarin + digoxin), lives alone, depression

2 Washington Bilateral TKA + wound dehiscence 13 SN, PT, OT, HHA Bilateral surgical wounds, MRSA risk, opioid management, morbid obesity, DVT prophylaxis

3 Tanaka-Morrison Left MCA CVA 11 SN, PT, OT, SLP, MSW, HHA Hemiplegia, aphasia, dysphagia, aspiration risk, neglect, elderly husband caregiver, bilingual

4 Constantinescu Post-liver transplant 19 SN, PT, OT, MSW, HHA Immunosuppression, PICC/IV abx, MRSA, NODAT, CKD4, AUD history, psych comorbidity, elderly unreliable caregivers

5 Alvarez-Jackson SBS + ECF + TPN 27 SN, PT, OT, MSW, HHA, RD Hickman, wound VAC, fistula, TPN, IVDU/methadone, 2 young children, Spanish-only caregiver, anticoagulation bridge

6 Thompson C4-C5 SCI, vent-dependent 32 SN, PT, OT, SLP, MSW, HHA, RT Quadriplegia, ventilator 24/7, trach, PEG, neurogenic bladder/bowel, autonomic dysreflexia, pressure ulcer, suicide risk, caregiver burnout

7 Reyes HLHS/Fontan failure, transplant eval 42 SN, PT, OT, SLP, MSW, HHA, RT, RD Single ventricle, home milrinone, PD, BiPAP, J-tube, plastic bronchitis, PLE, Fontan liver, CKD5, sickle trait, HIT, ASD/ID, non-verbal, conservatorship, 10 specialists, multi-agency coordination, caregiver burden (caring for patient + demented grandmother), cultural/linguistic, transplant evaluation

There you go — 7 evaluations escalating from high acuity to "this chart just set off the sprinklers in the compliance office." Every one is patient-side complete, office fields left blank, fully detailed, and audit-ready.

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thank you need another 7 but everything as a phone transcript completely different patients and make the format different

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CARE INDEED HOME HEALTH — INTAKE PHONE TRANSCRIPTS

7 Complete Patient Referrals via Telephone Intake

Format: Verbatim Transcripts with Inline Clinical Documentation

Patient / Representative Signature

Date

RN Clinician Signature

Date / Time